



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض ربيعاً : رشا عبدالموجود عبدالجواد الدبراني

العنوان : =

المحافظة : الجيزة

تاريخ تسجيل الإستمارة : 28-09-2026

رقم البطاقة : 28203052100643 رقم التليفون : 01151478370

العمر (تاريخ الميلاد) : 05-03-1982 المهنة : ربه منزل

الإقرار

أقر أنا الموقع أدناه أنني لا اتمتع بأي نظام علاج مثل:- التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات أو أي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإحصائي الاجتماعي (.....) التوقيع: (.....)

المقر بما فيه (.....) التوقيع: (.....)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : أبحاث أورام مزيد من الأبحاث و الاشعات [7954] قسم العلاج : الاورام

التعليق على التشخيص :

تقرير اللجنة الثلاثية المتخصصة

الاسم : د/محمد كيلاني الوظيفة : التوقيع :
الاسم : د/ندى حسن الوظيفة : التوقيع :
الاسم : د/نرمين رمضان الوظيفة : التوقيع :
أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقرر اللجنة بأن البيانات المدونة في هذا النموذج صحيحة.

تعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

..... د/..... د/..... د/..... د/.....

قرار اللجنة

التشخيص النهائي :
خطة العلاج :
المستشفى :
قسم :
جهة العلاج :
نفقات :
ملاحظات :

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

Name: رشا عبدالموجود عبدالجواد الديوان
National ID: 28203052100643



تقرير طبي

السيد

تحية طبية وبعد,,

بشأن تقرير طبي للمريض

إيماء للطلب المقدم من

رقم طبي:

بقسم:

نتشرف بأن نوضح فيما يلي تقريراً عن حالته

تاريخ ومساءة الخروج :
PM 01:17 05-07-2026

تاريخ ومساءة الدخول :
-- --:-- yyyy-mm-dd

وبعد الكشف والفحوصات والمتابعة وجد أنه

المریضة تعاني من ورم غير حميد بالثدي و تحتاج الي Further investigations and imaging

الوقت :

-- --:--

التاريخ :

05-07-2026

الطبيب المعالج :
د/شيماء لاشين

جمهورية مصر العربية
بطاقة تحقيق الشخصية

رشنا
عبدالموجود عبدالجواد الدبراني
قرية منشأة البكرى
مركز كرداسة - الجيزة

٢٨٢٠٣٠٥٢١٠٠٦٤٣

١٩٨٢/١٠

JB5652712

٢٠٢١/١٠ ٢٨٢٠٣٠٥٢١٠٠٦٤٣

ربة منزل

أنشى
معلمة
متروجة
خالد ابراهيم خضير بكوى

٢٠٢٨/١٠/٠٥ البطاقة مارية حتى

١

جمهورية مصر العربية
بطاقة تحقيق الشخصية

رشنا
عبدالموجود عبدالجواد الديراني
قرية منشأة البكرى
مركز كرداسة - الجيزة

٢٠٢١/١٠/٠٥ ٢٨٢.٣.٥٢١.٠٠٦٤٣

JB5652712

٢٠٢١/١٠ ٢٨٢.٣.٥٢١.٠٠٦٤٣

ربة منزل

أنثى
مستلمة
متزوجة
خالد ابراهيم خضير يلى

البطاقة سارية حتى ٢٠٢٨/١٠/٠٥

١

Name:	رشا عبدالموجود عبدالجواد الدبراني	
MR. :	10059	
Detailed Medical Report		
		السيد
تحتية طبية وبعد,,		
	بشأن تقرير طبي للمريض	إيماء للطلب المقدم من
	رقم طبي:	بقسم:
ننشر بأن نوضح فيما يلي تقريراً عن حالته		
	تاريخ وساعة الخروج :	تاريخ وساعة الدخول :
وبعد الكشف والفحوصات والمتابعة وجد انه The patient is a 44-year-old individual with a newly diagnosed invasive ductal carcinoma, estrogen receptor (ER) 8/8, progesterone receptor (PR) 8/8, HER2 positive, and Ki-67 approximately 30 %. Imaging demonstrated a right breast mass with ipsilateral lymph node involvement, clinically staged as T4b N1 M0. A bone scan was ordered and a contrast-enhanced spectral mammography (CESM) was requested to assess the contralateral breast. The oncology assessment confirmed right-sided breast cancer, clinical stage T4b N1 M0, with CESM showing a mass and non-mass enhancement. Physical examination revealed right breast redness, edema, and right axillary lymphadenopathy. Baseline neutropenia was noted (TLC $2.4 \times 10^9/L$, ANC $1.1 \times 10^9/L$). The treatment plan was initiated with a 20 % dose reduction of docetaxel (Taxotere) 110 mg and carboplatin AUC 4, with prophylactic pegfilgrastim (Neupogen) and dose escalation pending tolerability. Approval for Phesgo was pending. Laboratory results were reviewed and deemed acceptable. The plan was to continue the current regimen and administer a single dose of pegfilgrastim for cycle 2 (C2) of the taxane-carboplatin (TC) regimen. A repeat request for Phesgo approval was submitted after a recent echocardiogram; approval remained pending. Pharmacy documentation recorded the patient's weight at 78 kg, height 150 cm, body surface area $1.8 m^2$, and adjusted body weight 58.5 kg. The patient received cycle 2 (C2) of Taxotere 110 mg plus carboplatin AUC 4. During cycle 3 (C3), the patient received Taxotere 110 mg and carboplatin AUC 4, with supplemental treatment including G-CSF (one ampoule post-procedure). Laboratory values were acceptable. Additionally, the patient received the loading dose of Phesgo 1200 mg/600 mg subcutaneously (C1) and zoladex 10.8 mg subcutaneously. For cycle 3, the patient continued Taxotere and carboplatin AUC 4 and received the second dose of Phesgo 600 mg. Laboratory results were acceptable, but the patient experienced grade 3 diarrhea. The patient developed grade 2 diarrhea and an inflamed port-a-cath. Neutropenia persisted, prompting postponement of the next chemotherapy administration by one week while supportive laboratory monitoring was performed. The port-a-cath culture grew no organisms, but the site remained inflamed with an open wound. An external CBC showed persistent neutropenia (ANC $0.7 \times 10^9/L$) after one dose of G-CSF. Repeat CBC demonstrated improvement (TLC $5.10 \times 10^9/L$, ANC $3.7 \times 10^9/L$). The plan was to proceed with cycle 5 of the taxane-carboplatin (TC) regimen, administer three prophylactic doses of pegfilgrastim, and give the third dose of Phesgo. The patient has now completed five cycles of the TC regimen.		
تحريراً في ,		
	الوقت :	التاريخ : 18-09-2026
هنا أسامه عبد الرحيم عبد البر		الطبيب المعالج :
يعتمد ,		

05/11/2026

MDT FORM

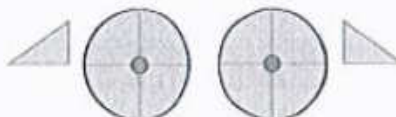
Name:	ترشا عبد المجيد عبد الجواد الحارثي	MRN:	10059
Age:	44 Y	ID:	28203052100643
Menopausal Status:	pre	Phone:	01151478370
NRS SCORE:		Marital Status:	married
BMI:	26	Family History:	-ve
CO Morbidity:	medically >> bronchial asthma, VERTIG	Reason Of MDT:	new case

Clinical Data:

BX:	IDC G II
ER	7.8
PR	8.8
HER	+3
KI67	30

R

L



TNM

Sonomammography:

SNG >> ACR C
IRREGULAR MASS RT BREAST LIQ MEASURING
4.7X3 CM AT 2-5 O CLOCK
REACHING TO NIPPLE WITH NIPPLE RETRACTION
AND DIMPLING OF THE OVERLING SKIN
SOLITARY PATHOLOGICAL LN

CESM >> LIQ MIDDLE POSTERIOR THIRD
IRREGULAR MASS 3.2 X 4.5 X 3 CM REACHING
NIPPLE WITH SKIN RETRACTION AND DIMPLING 5
SMALL SIMILAR SATALITTE ANTERIOR AND
POSTERIOR THIRD LOWER OUTER AND UPPER
OUTER MEASURING 9.1 X 6.7 X 4.5
REACHING THE NIPPLE
LEVEL 1 ENLARGED PAHTOLOGICAL LN

CLINICALLY
>> IBC , PAEU D CRAGNGE

Mets Workup:

CT CAP >> FREE
BONE SCAN >> FREE

History:

Recommendations:

FOR MACT

MDT Outcome & Plan

TCHP X 6 CYCLES THEN MRM

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
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Presidential Initiative for Women's Health
Pathology Lab - Cairo University Hospitals

المبادرة الرئاسية لصحة المرأة - معمل الباثولوجي
مستشفيات جامعة القاهرة



PATHOLOGY REPORT

Name: رشا عبد الموجود عبد الجواد
Patient's I.D : 28203052100643
Age: 44 Y.

Pathology Number: 45 /2026

Specimen Received Data: 23 / 02 /2026

Date Report Issued: 02/ 03 /2026

Clinical data: Right breast LIQ mass (BIRADS 5).

Nature of the specimen: U.S. Guided Tru Cut Biopsy.

GROSS EXAMINATION: Tissue core biopsy, totally submitted.

MICROSCOPIC EXAMINATION: Serial sections examined revealed cores of malignant tumor formed of groups, cords and tubules of moderately anaplastic ductal epithelial cells. Surrounding desmoplastic stroma with few TILs was seen. Tiny skin fragment was also seen.

IMMUNOHISTOCHEMISTRY PROCEDURE: Sections from paraffin block were treated with antigen retrieval solution and then treated with monoclonal antibodies for estrogen, progesterone, Her-2/neu AND Ki-67 (CONFIRM™ anti-Estrogen Receptor (ER) (SP1) Rabbit & CONFIRM™ anti-Progesterone Receptor (PR) (1E2), CONFIRM™ anti-HER-2/neu (4B5) Primary Antibody), and CONFIRM anti-Ki67 (30-9) Primary Antibody), using Ventana iView™ system. DAB was used as a chromogen and hematoxylin as a counter stain.

IMMUNOHISTOCHEMISTRY RESULTS:

Examination of section treated for estrogen receptors revealed strong positive nuclear staining in about 50% of tumor cells by chromogen.

Examination of section treated for progesterone receptors revealed strong positive nuclear staining in most of tumor cells by chromogen.

Examination of section treated for Her-2/neu revealed strong complete membrane staining in >10% of tumor cells by chromogen (score 3+).

Examination of section treated for Ki67 revealed positive nuclear stain in average of $\pm 25-30\%$ of tumor cells.

DIAGNOSIS: Right Breast Mass, U.S. Guided Tru Cut Biopsy:

- Invasive Duct Carcinoma, NST, Grade II.
- **Positive** For Estrogen Receptors, Score 7/8.
- **Positive** For Progesterone Receptors, Score 8/8.
- **Positive** For Her-2/neu Over-Expression (Score 3+).
- $\pm 25-30\%$ Ki67 Proliferative index.

DR. MIRAN ATIF

DR. SARA ABOU-FANDOUD

د. سارة أبو فندود
د. ميران اتيف

Name: رشا عبد الموجود عبد الجواد	Date: 2/22/2026
Age: 44 Years	ID: 28203052100643

**Full Field Digital Mammography With Complementary
High Resolution Ultrasound Examination**

Indication: *right breast lump.*

Findings:

- Heterogeneously dense breasts (ACR c).
- An irregular shaped spiculated dense mass lesion is seen at right breast lower inner quadrant which by ultrasound corresponds to an irregular shape hypoechoic solid mass lesion extending from 2 to 5 o'clock measuring about 4.7x3cm and is seen reaching to the nipple with subsequent nipple retraction. Associated tethering/dimpling of the overlying skin is also noted with shortest distance to skin is 0.6cm.
- No left breast spiculated masses, clustered microcalcifications or areas of architectural distortion seen.

Complementary US:

- The left breast parenchyma appears homogenous with no solid or cystic lesions.
- Normal contour and skin thickening of left breast.
- Normal left nipple /areola complex.
- Left nonspecific axillary lymph nodes with preserved fatty hilum.
- Solitary pathologically enlarged right axillary lymph node is seen showing focal cortical thickening of 0.5 cm and eccentric hilum.

Opinion :-

- *Right breast LIQ malignant looking mass lesion with ipsilateral pathological axillary lymph node (BIRADS 5), for histopathology.*
- *Normal sonomammography study of left breast (BIRADS 1).*

Thanks for referral,

Dr.Eman Badawy, MD



PR

Name: RASHA ABD EL MAOGOUD ABD EL GAWAD	Date: 12/03/2026
Age: 44 Years	ID: 100395211

Multislice post contrast CT scan of the abdomen and pelvis with multiplanar reformatted images revealed:

****Reported history of breast IDC.**

- Average hepatic size showing homogenous parenchymal pattern. No hepatic lesions or biliary radicles dilatation.
- Normal CT appearance of the spleen, pancreas, suprarenal glands, aorta, IVC and pelvic organs.
- Normal size and excretory capacity of both kidneys.
- No retro-crural, para-aortic or pelvic lymphadenopathy.
- No ascites.

OPINION:

- No evidence of hepatic metastatic deposit.
- unremarkable CT of the abdomen and pelvis.

Dr.Naglaa Mahmoud Ali MSC
Dr. Rasha Mostafa,MD



Patient Name	رشا عبدالمجود عبدالحواد الدتراني	PID	1021012607 0874	D/T	06-04-2026
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C.T. SCAN OF THE CHEST WITH CONTRAST:-

Axial contiguous thin cuts were taken through the chest with I.V. contrast injection.

Recently diagnosed case of breast cancer. Initial study for staging.

- Mild bilateral lower lobe mosaic parenchymal attenuation.
- No evidence of abnormal masses, nodules or consolidative patches.
- Normal C.T. appearance of the mediastinum vessels.
- No sizable mediastinal or hilar adenopathy.
- Patent tracheo bronchial tree with no abnormal bronchial dilatation.
- No pleural or pericardial effusion.
- Normal cardiac borders and chambers.
- Right breast skin thickening reaching 15mm, with a suspicious soft tissue mass measuring around 12.5x22mm.
- Multiple right axillary and interpectoral LNs, most of them are rounded with lost hilum yet subcentimetric, the largest measures around 8mm.
- Scanned bone window reveals no suspicious lytic or sclerotic lesions.

IMPRESSION:-

- No suspicious pulmonary nodules.
- Right breast suspicious mass, skin thickening and axillary LNs for correlation with sonomammography.

Shohenda Magdy, MSc

1-09

Patient ID: 94713	Patient Name English: Rasha Abda/mawgod Abdgawad
مرحلة ١ شهر	2026/04/16 01:13:33 PM :Report Date

Contrast enhanced Spectral Mammography

CLINICAL SUMMARY: 44 years old female patient with pathologically proven invasive ductal carcinoma grade II (+ve ER, +ve PR, +ve HER2 and Ki67= 20-30 %)

CC & MLO low energy & recombined (post contrast) images

FINDINGS:

Background Uptake: Minimal background enhancement.

ACR: C (heterogenous dense)

Right breast:

- **Low energy (Non contrast) images:** Right breast shows middle/posterior 1/3 LIQ irregular shaped spiculated dense lesion associated with architectural distortion.
- **Recombined (Post contrast) images:**
 - Right breast shows focal malignant process inform of large LIQ middle/posterior 1/3 irregular spiculated heterogenous moderate mass enhancement and measuring about 3.2 x 4.5 x 3.0 cm at its AP, TS and CC diameters and reaching nipple with skin retraction and dimpling (**Index lesion, 3-6 ° clock position**).
 - **Multiple at least five small similar satellites** are noted at the anterior and posterior 1/3s LOQ and UOQ and adjacent to the index lesion and collectively measuring overall about 9.1 x 6.7 x 4.5 cm at its AP, TS and CC diameters and reaching nipple.
 - The most anterior satellite measures about 5.0 x 4.8 mm and about 1 cm from the nipple and about 5.3 cm from index lesion and located at 10:00 clock position.
 - The most posterior satellite measures about 20 x 17 mm and about 7 cm from the nipple and about 3.6 cm from index lesion and invading the underlying muscles.
 - The intervening area between the masses showing non mass enhancement.
- **Complementary US:**
 - The right breast shows few interconnected irregular shaped spiculated hypoechoic lesions located at 3:00-6:00 clock position with skin dimpling and nipple retraction.
 - Few small similar satellites are noted posterior and lateral to index lesion located at 3:00-6:00 and 10:00 clock position.
 - Few enlarged axillary lymph node at level I with focal cortical thickening, largest one measuring about 18.2 x 14.6 mm with focal cortical thickening measuring about 5.4 mm.

Left breast:

- **Low energy (Non contrast) images** No asymmetry or distortion or suspicious microcalcifications.
- **Recombined (Post contrast) images:** no pathological mass or non-mass enhancement.
- Normal skin / nipple / areolar complex.
- **Complementary ultrasound:**
 - No enlarged axillary LNs.

IMPRESSION:

- **RIGHT BREAST:** (BIRADS 6= pathological proven) Right locally advanced multiple UIQ and UOQ masses and non-mass enhancement as described above.
- **LEFT BREAST:** (BIRADS 2) normal study.

Best regards,
Dr. Hedaya Nour, MD

Name : Rasha Abdel Mawgood Abdel Jawad Al-Debarani Date : 24-04-2026 08:39
Branch : Dokki Medical No. : 358702
Ref.Physician Prof : Goustav Roussey Egypt Ins.Company : تعاقد مستشفى

Tc^{99m} MDP SKELETAL SCINTIGRAPHY

History

43 year-old female patient , presented with history of right breast cancer and referred for skeletal survey .

Technique

- Anterior and posterior whole body scans were acquired 2 hours post injection of 20 mci of the tracer.

Findings

- No evidence of tracer uptake abnormalities that would account for bone secondaries could be seen.
- Degenerative changes are noticed at different levels of the spine and around big joints.

IMPRESSION

- No definite osseous metastases detected on scintigraphic basis.

Much obliged
Dr. Alaa Nagy, MD
Nuclear medicine

PATHOLOGY REPORT

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Patient's I.D : 28203052100643

Age: 44 Y.

Pathology Number: 45 /2026

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Date Report Issued: 02/ 03 /2026

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- Positive For Her-2/neu Over-Expression (Score 3+).
- $\pm 25-30\%$ Ki67 Proliferative index.

DR. MIRAN ATIF
DR. SARA ABOU-FANDOUD

د. ميران اتيف
د. سارة أبو فندود



Name: RASHA ABD EL MAOGOUD ABD EL GAWAD

Age: 44 Years

Date: 12/03/2025

ID: 106395211

Multislice post contrast CT scan of the abdomen and pelvis with multiplanar reformatted images revealed:

****Reported history of breast IDC.**

- Average hepatic size showing homogenous parenchymal pattern. No focal lesions or biliary radicles dilatation.
- Normal CT appearance of the spleen, pancreas, suprarenal glands, and pelvic organs.
- Normal size and excretory capacity of both kidneys.
- No retro-crural, para-aortic or pelvic lymphadenopathy.
- No ascites.

OPINION:

- No evidence of hepatic metastatic deposit.
- unremarkable CT of the abdomen and pelvis.

Dr. Naglaa Mahmoud Ali MSC
Dr. Rasha Mostafa, MD

Patient Name	رنا عبدالرحمن عبدالرحمن الشربشي	PID	1021012607 0874	D/T	06-04-2026
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CT SCAN OF THE CHEST WITH CONTRAST:-

Axial contiguous thin cuts were taken through the chest with I.V. contrast injection.

Recently diagnosed case of breast cancer. Initial study for staging.

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IMPRESSION:-

- No suspicious pulmonary nodules.
- Right breast suspicious mass, skin thickening and axillary LNs for correlation with sonomammography.

Shohenda. Magdy, MSc

Email: feedbackradiology@gmail.com

مستشفيات جامعة عين شمس - شارع رمسيس - الطبعة الثامنة

Patient ID: 94713	Patient Name English: Rasha Abdalmawgod Abdgawad
٩٤٧١ ، رشا رافع Age:	2026/04/16 01:13:33 PM :Report Date

1-02

Contrast enhanced Spectral Mammography

CLINICAL SUMMARY: 44 years old female patient with pathologically proven invasive ductal carcinoma grade II (+ve ER, +ve PR, +ve HER2 and Ki67= 20-30 %)

CC & MLO low energy & recombined (post contrast) images

FINDINGS:

Background Uptake: Minimal background enhancement.

ACR: C (heterogenous dense)

Right breast:

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Left breast:

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- Normal skin / nipple / areolar complex.
- **Complementary ultrasound:**
 - No enlarged axillary LNs.

IMPRESSION:

- **RIGHT BREAST:** (BIRADS 6= pathological proven) Right locally advanced multiple LIQ and UOQ masses and non-mass enhancement as described above.
- **LEFT BREAST:** (BIRADS 2) normal study.

Best regards,
Dr. Hedaya Nour, MD

Name : Rasha Abdel Mawgood Abdel Jawad Al- Debarani Date : 24-04-2026 08:39

Branch : Dokki

Medical No. : 358702

Ref.Physician Prof : Goustav Roussey Egypt

تعاقد مستشفى : Ins.Company :

Tc^{99m} MDP SKELETAL SCINTIGRAPHY

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- No definite osseous metastases detected on scintigraphic basis.

Much obliged
Dr. Alaa Nagy, MD
Nuclear medicine